



The Havana Specialist Hospital Board and Committee Handbook

The board's standing reference: how we govern, and how each of us does our part

Prepared for: The Board, Committees, and Management of Havana Specialist Hospital Limited

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Companion to: the Board Operating Model, the Committee Terms of Reference, the Delegation of Authority schedule, the Director Guide, and the Governance Training Curriculum

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This handbook is the one document a director, a committee chair, the Company Secretary, or a member of management can pick up and find how the HSH board actually works, and what is expected of them in it. It gathers the operating model, the committees, the duties, the conduct, the roles, the instruments, and the practical playbooks into a single reference. It is not a substitute for the Operating Model or the Terms of Reference, which remain the governing documents; it is the working manual that makes them usable day to day. Read the part you need, keep the whole to hand, and update it as the board learns.

How to use this handbook

The handbook is in eight parts and a set of appendices.

- **Part A, The institution and the architecture.** What HSH is, who owns it, and the shape of its governance.
- **Part B, The legal foundation.** The company, CAMA 2020, directors' duties, and the reserved matters and delegation that flow from them.
- **Part C, How the board works.** The cycle, the pack, decisions, and resolutions.
- **Part D, The committees.** The five board committees and the Afya-HSH Governance Committee, each in full.
- **Part E, Conduct.** Conflicts, related-party dealings, confidentiality, fair process, and the code of conduct.
- **Part F, The roles.** What each seat at and around the table is for.
- **Part G, The instruments.** The templates and records, and how to use them.
- **Part H, Playbooks.** Step-by-step guides for the recurring situations.
- **Appendices.** Glossary, a CAMA quick reference, the governance calendar, the register index, and a decision-rights summary.

Directors new to a board should read the Director Guide first, then keep this handbook as the reference behind it.



Part A, The institution and the architecture

A1. What Havana Specialist Hospital is

Havana Specialist Hospital Limited is a private specialist hospital in Lagos, owned by two founding families and operated in partnership with **Afya Care**, which runs the hospital day to day. The board is being placed on a formal corporate-governance footing so that the hospital can grow, sustain itself with or without any single partner, and be governed fairly across the two families.

A2. The four core jobs of the board

Everything the board does reduces to four jobs. When in doubt about whether something is the board's business, test it against these:

1. **Set direction.** Agree the strategy, the annual business plan, and the budget.
2. **Oversee performance and risk.** Watch how the hospital and the Afya partnership are performing against the plan, and how the main risks are being managed.
3. **Hold management to account.** Require, read, and challenge reporting; ask for action where performance is off course.
4. **Protect the owners and the mission.** Keep the hospital viable, well-governed, and fair to both families, and safeguard the quality of care and the standing of the institution.

A3. Ownership and the board

The board is **eight directors, split four and four** between executive directors (drawn from the two families) and non-executive directors.

Executive directors:

- **Dr Ijeoma Iwuala**, Medical Director and executive director (Family A). Runs clinical operations; reports *in* to the board and its committees.
- **Ugo Nwokoro**, executive director (Family A), product and digital.
- **Nwando Okeke**, executive director (Family B), legal.
- **Okeke Chukwuemeka**, executive director (Family B), operations.

Non-executive directors:

- **Dennis Olisa**, interim Board Chair (independent; finance and audit background).
- **Chief Charles Odunukwe**, independent (corporate and investment; trusted across both families).
- **Prof Oliver Ezechi**, independent and Lead Independent Director (clinical and academic; trusted across both families).
- **Dr Debo Odulana**, independent (Consult for Africa adviser; related-party, disclosed).

A4. The governance architecture, at a glance

HSH runs a **two-tier** structure:

- The **board** meets quarterly, plus a strategy session, and holds the reserved matters.
- Beneath it sit **five board committees** that assure on the board's behalf and report to the board.
- The **Afya-HSH Governance Committee**, a joint HSH and Afya body, meets monthly to oversee the operating partnership, and reports up to the board. The **operational committees** (Quality Assurance,



Hospital Manager, Clinical Administrator, Performance Review) report through the Governance Committee, not directly to the board.

The single rule that keeps the architecture clean: **board committees assure on the board's behalf and report to the board; everything else is an operating committee that reports up through the Afya-HSH Governance Committee.**

Part B, The legal foundation

The legal content here is a working reference for directors, written plainly. It is not legal advice, and the exact statutory citations are confirmed by the board's legal advisers and by the governance-training faculty (Barr. Gesiye Otuogha and Barr. Osahon Omoruyi). Where a section number would be cited, the training materials carry it; this handbook keeps to the substance.

B1. The company as a separate legal person

HSH is a company incorporated under the Companies and Allied Matters Act 2020 (CAMA). It is a **separate legal person** from its owners: it owns its own property, enters its own contracts, and carries its own liabilities. The owners' liability is limited to their shares. This separation is why directors owe their duties to *the company*, not to the family that appointed them.

B2. The three organs of the company

- **The members (owners) in general meeting.** The families as shareholders, exercising the powers the law and the constitution reserve to them.
- **The board of directors.** Governs the company: sets direction, oversees, and holds management to account.
- **Management.** Runs the company day to day, here with Afya, under the Medical Director.

A healthy company keeps these three distinct. Owners do not manage; the board does not run operations; management does not set its own mandate.

B3. Directors' duties under CAMA 2020

Every director, executive or non-executive, owes the company the following duties. They are the frame around everything in this handbook.

- **Act in good faith, in the best interests of the company as a whole.** Not one family, not one department, not oneself.
- **Exercise powers for a proper purpose.** Use board power for the reason it was given, not for advantage.
- **Exercise reasonable care, skill, and diligence.** Prepare, attend, pay attention, and apply judgement to the standard expected of someone in your position.
- **Avoid conflicts of interest**, and where one arises, declare and manage it (Part E).
- **Do not make secret profit or accept improper benefit** from the office.
- **Have regard to the wider interests** the law requires, including the workforce and the community the hospital serves.



Directors who prepare, attend, apply honest judgement, and declare their conflicts are doing exactly what the law asks. The duties are demanding but entirely meetable by a diligent person.

B4. Liability, and the everyday conduct that protects you

Directors carry real duties, and in defined circumstances can be personally exposed, for breach of duty, or for reckless or fraudulent trading. The protection is not secrecy or absence; it is diligence: come prepared, act in good faith, declare interests, keep to your powers, and rely on proper information. The board should also put appropriate **directors' and officers' insurance** and indemnities in place. This is standard and the People and Remuneration Committee should confirm it is arranged.

B5. The two-family dimension and minority protection

CAMA protects owners against conduct that is **unfairly prejudicial** to them. In a two-family company this is not abstract: it means the board must treat both families fairly in substance and in process. Fair process (Part E) is therefore not a courtesy; it is how the board stays lawful and legitimate toward both sides.

B6. Reserved matters and the Delegation of Authority

The board keeps certain decisions for itself, the **reserved matters**, and delegates the rest downward through the **Delegation of Authority (DoA) schedule**, which says which decisions belong to the board, to a committee, to the Afya-HSH Governance Committee, and to management, and at what financial thresholds. The DoA is the single source of truth for "who decides this." When unsure, check it. Reserved matters typically include strategy and budget approval, major capital and investment, changes to the Afya arrangement, senior appointments and remuneration frameworks, and anything touching the company's constitution or the owners' rights. The governing detail lives in the DoA schedule.

Part C, How the board works

C1. The annual cycle

The board works to a **rolling twelve-month calendar** that sequences the committees to feed the board in the right order: committees meet, examine their areas in depth, and report up, so that by the time a matter reaches the full board it has usually been worked through by a committee. The board meets **quarterly**, with an annual **strategy session**. The calendar is maintained by the Company Secretary (Appendix 3).

C2. The board pack

The board works from a **single consolidated pack**, circulated at least five working days ahead. It contains: the agenda, the chair's notes, the management report, the committee reports, the risk summary, the finance pack, and the decisions log. The pack is built on standard templates so it is consistent meeting to meeting, and it reports **by exception**, highlighting what is off target rather than restating everything.

The single most important director habit: **read the pack before the meeting and note your questions**. A board that reads its pack governs; one that does not, drifts.

C3. Making decisions

- **Reporting by exception** brings the off-target items to the board's attention; the board spends its time on those and on the decisions reserved to it.



- **Decisions are minuted with their reasons**, so those who disagreed understand why, and so the record stands.
- **Fair process** (Part E) governs how the board reaches decisions: everyone heard, reasons explained, decisions for the company, and support once decided.

C4. Resolutions and the mechanics of a valid decision

- A meeting needs proper **notice**, a **quorum**, and a chair.
- Most decisions are **ordinary resolutions** (a simple majority). Certain matters, defined by law or the constitution, need a **special resolution** (a higher threshold). Some decisions may be taken as **written resolutions** without a meeting where the rules allow.
- The **minute book** is a legal record; the Company Secretary keeps it. A decision not properly minuted is a decision the board cannot later rely on.

Module M4 of the training teaches these mechanics in depth; the Company Secretary is the standing guardian of them.

Part D, The committees

Each committee is a committee **of the board**, with delegated authority to oversee and assure, and reports to the board on a one-page template (key matters, decisions taken, items escalated, decisions requested). The Company Secretary services every committee, circulates papers at least five working days ahead, and keeps minutes and an action log. Any member with an interest declares it and recuses. Committees may take external professional advice at the company's expense within a board-approved limit. Terms of reference are reviewed annually. The full detail is in the Committee Terms of Reference; the summaries below are the working reference.

D1. Finance, Investment and Operations Committee

- **Purpose.** The board's *performance* committee: financial performance, capital and investment, and operational delivery. Distinct from the independent assurance of Audit and Risk.
- **Chair.** Co-chaired across the families: **Okeke Chukwuemeka** (Family B) and **Ugo Nwokoro** (Family A). A matter the co-chairs cannot agree goes to the full board, not to a casting vote.
- **Members.** Chief Odunukwe (investment), a further NED (Dr Odulana or another). In attendance: Hospital Manager / Head of Finance, Head of Operations, Afya finance lead.
- **Quorum.** Two members, at least one non-executive. **Cadence.** Quarterly.
- **Does.** Management accounts and budget monitoring; cash and working capital; capital appraisal and post-investment review; operational KPIs; the commercial performance of the Afya arrangement; pricing, payer mix, and revenue.

D2. Audit and Risk Committee

- **Purpose.** Independent assurance over financial reporting, internal control, audit, the enterprise risk register, and compliance. Sits apart from the finance function it holds to account.
- **Chair.** **Chief Charles Odunukwe** (independent, financially literate; deliberately not the Board Chair and not the Finance chair).



- **Members.** Nwando Okeke (legal, compliance, related-party), Dr Odulana (recused from Consult for Africa related-party items). In attendance: Head of Finance, internal and external audit.
- **Quorum.** Two members; the Board Chair and executives attend only by invitation. **Cadence.** Quarterly plus an audit-planning meeting (five a year).
- **Does.** Annual financial statements and going-concern; internal control and internal audit; external audit appointment, independence, and findings; the single enterprise risk register and risk appetite; related-party transactions involving the families and Afya; the independent read on Afya's financial delivery.

D3. Quality and Clinical Governance Committee

- **Purpose.** Assure the board that care is safe and high quality and that clinical risk is managed. A core, non-delegable board responsibility.
- **Chair.** Prof Oliver Ezechi (independent; also Lead Independent Director).
- **Members.** Dr Ijeoma Iwuala (Medical Director, as executive clinical lead reporting *in*), Dr Hanson if appointed (nursing, accreditation). In attendance: Clinical Administrator, Quality Assurance lead.
- **Quorum.** Two members, at least one non-executive. **Cadence.** Quarterly.
- **Does.** Clinical quality, patient safety, and outcomes; incident and near-miss reporting, investigation, and learning; oversight of the Tier 2 clinical functions; credentialing, protocols, and scope of practice; clinical risk (feeding the enterprise register owned by Audit and Risk); accreditation readiness.

D4. People and Remuneration Committee (with Nominations)

- **Purpose.** Board composition and succession, appointments, executive and director remuneration, board evaluation, workforce strategy, and conflicts management.
- **Chair.** Nwando Okeke (Family B). Chairing is given to the family that does *not* hold the Medical Director role, so the body controlling appointments and pay is a visible counterweight.
- **Members.** Prof Ezechi (LID), an executive director from the other family, Dr Odulana (recused from Consult for Africa fee matters).
- **Conflict guardrail.** Because the chair is an executive director, a hard recusal applies: the chair and any member step out of decisions on their own and close family's remuneration, and the independents present carry those items. Nominations may be led by the chair; sensitive remuneration decisions are taken by the independents.
- **Quorum.** Two members, at least one independent NED for any remuneration decision. **Cadence.** Twice a year plus as required.
- **Does.** The skills-gap review and appointment pipeline (including the payer-expertise and nursing decisions); induction, development, and annual board and chair evaluation (chair appraisal led by the LID); the executive and senior remuneration framework; the board sitting-allowance and participation policy; the conflicts and related-party register. It also confirms directors' and officers' insurance is in place.

D5. Transformation and Digital Committee (time-boxed)

- **Purpose.** Oversee the digital and operational transformation during the build phase.
- **Chair.** Dr Debo Odulana (independent), so the committee provides oversight rather than being run by the executive delivering the work.



- **Members.** Head of Operations, Afya technology lead, relevant executives in attendance; external adviser as needed.
- **Conflict guardrail.** The chair recuses from any decision where Consult for Africa or CadreHealth could deliver or bid for the work.
- **Quorum.** Two members, at least one non-executive. **Cadence.** Monthly while active, then quarterly.
- **Does.** Technology and digital roadmap and prioritisation; major system selection, implementation, and benefits realisation; data governance and information security; digital patient experience.
- **Sunset.** Constituted with a defined review or sunset date; expected to migrate to a management function once transformation stabilises. It should not become a permanent board committee by default.

D6. The Afya-HSH Governance Committee (joint operating body)

- **Purpose.** Oversee the day-to-day operating partnership between HSH and Afya. Monthly. It sits *beneath* the board and reports up.
- **Reports up.** The operational committees, Quality Assurance, Hospital Manager, Clinical Administrator, and Performance Review, report *through* this committee, not directly to the board.
- **The independence rule.** Because this committee is run *jointly with Afya*, the board cannot rely on it alone to grade Afya's own delivery. The board keeps an independent line of sight through Audit and Risk (on the numbers) and Quality and Clinical Governance (on care), plus a standing quarterly partnership review at the full board. This protects HSH's ability to thrive with or without Afya.

Part E, Conduct

E1. Conflicts of interest

Conflicts are normal in a two-family, partner-operated, adviser-supported company. What matters is that they are handled openly.

- **Declare it.** If a matter touches you, your family, or a business you are connected to, say so at the start of the discussion.
- **Step out of it.** Where the interest is material, leave the room for that decision (recusal), so the others decide freely.
- **Record it.** The Company Secretary keeps the register of interests and notes recusals in the minutes.

This applies to everyone, including the independent directors and advisers. Declaring a limit on your own involvement is a sign of a trustworthy director, not a weakness. Worked recusals already built into the model: Dr Odulana on Consult for Africa fee and related-party items; the People and Remuneration chair and members on their own family's remuneration; the Transformation chair on any work Consult for Africa or CadreHealth could deliver.

E2. Related-party transactions

Dealings with the families, with Afya, or with directors' connected businesses are **related-party transactions** and get special handling: declared, reviewed by Audit and Risk within a board-approved threshold (larger ones referred to the board), priced and documented at arm's length, and disclosed. The register is maintained by the Company Secretary and overseen by People and Remuneration.

E3. Confidentiality



What is said in the boardroom stays there. Board and committee papers are confidential. Confidentiality is what lets directors speak candidly and challenge freely; it is not optional.

E4. Fair process, the engine of a two-family board

The board's legitimacy rests on *how* it decides as much as *what* it decides.

- **Everyone is heard before a decision is made.** People accept outcomes they did not prefer when they had a genuine voice.
- **Reasons are explained**, so those who disagreed understand why.
- **Decisions are for the company**, not for a side. Return always to: what is best for HSH?
- **Once decided, support it.** A decision properly reached, with everyone heard, is the board's decision, and the board stands behind it together.

E5. The code of conduct, in one place

A director: comes prepared; acts in good faith for the company; exercises care, skill, and diligence; declares and manages conflicts; keeps board matters confidential; acts within their powers per the DoA; disagrees on substance, not personally; and supports properly-made decisions. That is the whole of it.

Part F, The roles

- **Board Chair (Dennis Olisa, interim).** Leads the board, sets its tone and agenda with the Company Secretary, ensures fair process, and represents the board. Chairs no committee, so committee assurance stays independent of the chair. The appointment is interim and reviewed through an open process the board owns.
- **Lead Independent Director (Prof Oliver Ezechi).** A senior independent who balances the chair, leads where the chair has a conflict, and leads the chair's annual appraisal. Standard good practice.
- **Medical Director (Dr Ijeoma Iwuala).** The senior executive running clinical operations, and an executive director. Reports *in* to the board and committees; chairs none, because you cannot independently assure your own work. Sits on Quality and Clinical Governance as executive clinical lead, a position of real influence.
- **Executive directors.** Owners and executives who, in the boardroom, wear the board hat and act for the company as a whole. Several chair or co-chair a committee.
- **Non-executive directors.** Independent judgement, not in management. Chair the assurance committees.
- **Committee chairs.** Set the agenda, run the meeting, reach and record decisions, report to the board, and manage conflicts. A chair draws the work out of the committee; they are not the expert on everything in it.
- **Company Secretary.** The engine room of governance and the standing guardian of compliance: assembles and circulates the board pack, runs the calendar, keeps minutes, decisions, and the action log, maintains the statutory registers (members, directors, charges, persons with significant control, interests, related-party), administers the DoA, coordinates the committees and the Afya interface, and holds reporting to the templates. A statutory role under CAMA. Section 5 of the enablement proposal recommends the secretariat be given a dedicated owner rather than shared informally among directors.



Part G, The instruments

The board runs on a defined set of instruments. They are the dashboard that lets a quarterly board stay in control of a daily hospital.

Instrument	What it is	Owner
Consolidated board pack template	The single pre-read: agenda, chair's notes, management report, committee reports, risk summary, finance pack, decisions log	Company Secretary
Committee report templates	One-page report per board committee and the Governance Committee, common format (metrics vs target, exceptions, risks, decisions requested)	Committee chairs / Secretary
Agenda, minutes, action-log templates	Standardised across board and committees	Company Secretary
Reporting dashboard and KPI pack	The standing performance dashboard that feeds the board by exception	Management / Secretary
Board and committee annual calendar	Rolling twelve-month schedule sequencing committees into the board	Company Secretary
Delegation of Authority schedule	Who decides what, and at what threshold	Board / Secretary
Committee terms of reference	The mandate for each committee	Board
Registers	Members, directors, charges, persons with significant control, interests, related-party	Company Secretary

The rule: **use the templates**. They are what make reporting consistent, decisions traceable, and the board's time well spent. When a report arrives off-template, send it back.

Part H, Playbooks

Short, repeatable guides for the recurring situations.

H1. Inducting a new director (People and Remuneration + Company Secretary)

1. Issue the induction pack: constitution summary, strategy, latest accounts, the Afya agreement summary, the risk register, this handbook, and the Director Guide.
2. Company Secretary walks the new director through the pack, calendar, and minutes.
3. Assign the committee and, if chairing, walk the first agenda through with the Secretary.
4. Enrol in the next governance-training cohort.
5. Confirm directors' and officers' insurance covers them from appointment.



H2. Preparing for and chairing a committee meeting (committee chairs)

1. With the Secretary and relevant executives, set the agenda on what matters.
2. Confirm quorum and that papers went out five working days ahead.
3. Open by inviting declarations of interest; handle any recusal.
4. Run to time; draw out every voice; prevent domination; move to a clear decision.
5. Confirm and minute each decision *and its reasons*; capture actions and owners in the action log.
6. Prepare the one-page report to the board and flag decisions requested.

H3. Escalating a matter (management and committees)

1. Check the DoA: is this mine, a committee's, the Governance Committee's, or the board's?
2. If it exceeds your threshold or is a reserved matter, escalate to the next level with a clear recommendation, not just a problem.
3. Report material exceptions upward promptly; do not wait for the next scheduled meeting for a serious risk.

H4. Handling a serious clinical incident (Quality and Clinical Governance + Medical Director)

1. Immediate clinical management and patient safety come first, under the Medical Director.
2. Trigger the incident investigation per protocol; observe the duty of candour.
3. The Medical Director reports the incident *in* to the Quality and Clinical Governance Committee; the committee assures the investigation and the learning.
4. Escalate serious events to the board; take legal advice where indicated (the training's clinical-legal module covers this).
5. Feed the clinical risk into the single enterprise risk register held by Audit and Risk.

H5. The quarterly Afya partnership review (full board)

1. Take the Governance Committee's report *and* the independent reads from Audit and Risk (numbers) and Quality (care).
2. Assess Afya's delivery against the agreement's commitments.
3. Record the board's independent view, so HSH always knows, on its own account, whether the partnership is delivering.

H6. Managing a conflict or related-party matter (any body)

1. Declare at the start of the item.
2. If material, recuse: leave the room for that decision.
3. Secretary records the declaration and recusal in the minutes and the register.
4. For a related-party transaction, route through Audit and Risk within threshold, or the board above it; price and document at arm's length.



Appendices

Appendix 1, Glossary

- **Board / director.** The body that governs the hospital, and its members.
- **Executive director (ED).** A director who is also part of the executive or ownership.
- **Non-executive director (NED).** A director not part of management, brought in for independent judgement.
- **Lead Independent Director (LID).** A senior independent director who balances the chair and leads where the chair has a conflict.
- **Medical Director (MD).** The senior executive running clinical operations; here, also an executive director.
- **Governance vs management.** Governing is setting direction and holding to account (the board); managing is running the hospital day to day (management and Afya).
- **Committee.** A smaller group of directors that examines an area in depth and reports to the board.
- **Board pack.** The consolidated papers circulated before each meeting.
- **Reporting by exception.** Reporting that highlights what is off target rather than restating everything.
- **Delegation of Authority (DoA).** The schedule of who decides what, and at what threshold.
- **Reserved matters.** The big decisions the board keeps for itself.
- **Recusal.** Stepping out of a decision in which you have a conflict.
- **Fiduciary duty.** The legal duty to act in the best interests of the company as a whole.
- **Related-party transaction.** A dealing with a family, Afya, or a director's connected business, handled under special rules.
- **Ordinary / special resolution.** A decision by simple majority / by a higher threshold set in law or the constitution.
- **Persons with significant control (PSC).** Those who ultimately own or control the company, recorded in a statutory register.
- **Afya-HSH Governance Committee.** The joint HSH and Afya body overseeing the operating partnership, reporting up to the board.

Appendix 2, CAMA 2020 quick reference for directors

A plain-language checklist of what the Act asks of a director and the company. Exact section numbers are carried in the training materials and confirmed by the legal faculty.

- **Duties:** good faith and best interests of the company; proper purpose; care, skill, and diligence; avoid conflicts; no secret profit or improper benefit; regard to wider interests.
- **Interests:** declare any interest in a contract or arrangement; observe the rules on loans to directors and related-party dealings.
- **Meetings:** proper notice, quorum, and minutes; the right resolution type for the matter.
- **Records:** keep the statutory registers (members, directors, charges, PSC, minute book) and the register of interests.
- **Reporting and filing:** prepare financial statements, arrange audit as required, and make annual returns and CAC filings on time.



- **Owners' protection:** avoid conduct unfairly prejudicial to any owner, a live duty in a two-family company.
- **Distress:** understand the exposure for reckless or fraudulent trading and the going-concern duty.

Appendix 3, The governance calendar (indicative)

Quarter	Board	Committees feeding it
Q1	Board meeting + Afya review	Finance, Audit and Risk (incl. audit planning), Quality, Transformation (monthly while active)
Q2	Board meeting + Afya review	Finance, Audit and Risk, Quality, People and Remuneration, Transformation
Q3	Board meeting + strategy session	Finance, Audit and Risk, Quality, Transformation
Q4	Board meeting + Afya review	Finance, Audit and Risk (year-end and statements), Quality, People and Remuneration (evaluation), Transformation

The Company Secretary maintains the live calendar and sequences committees to feed each board meeting in the right order.

Appendix 4, Register index (kept by the Company Secretary)

Register of members; register of directors and secretaries; register of charges; register of persons with significant control; register of directors' interests; related-party register; minute books (board and each committee); the decisions and action log.

Appendix 5, Decision-rights summary

The DoA schedule is the governing document; this is the orientation.

Decision type	Typically sits with
Strategy, annual plan, budget approval	Board (recommended by Finance)
Major capital / investment above threshold	Board (appraised by Finance)
Operational and capital spend within threshold	Finance, Investment and Operations Committee / management per DoA
External auditor, risk appetite, related-party above threshold	Audit and Risk / Board
Clinical quality standards, credentialing, incident assurance	Quality and Clinical Governance
Director appointments, chair, remuneration framework	People and Remuneration → Board
Technology roadmap and major system selection	Transformation and Digital → Board
Day-to-day operations	Management and Afya, within the DoA



Decision type	Typically sits with
Changes to the Afya arrangement, the constitution, owners' rights	Board (reserved matter)

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